

Dobutamine: Drug information - UpToDate

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For additional information see "Dobutamine: Patient drug information" and "Dobutamine: Pediatric drug information"

For abbreviations, symbols, and age group definitions show table

Pharmacologic Category

- Adrenergic Agonist Agent;
- Inotrope

Dosing: Adult

Acute decompensated heart failure

Acute decompensated heart failure: Note: May consider for short-term use in patients with low cardiac index and hypotension or end-organ hypoperfusion (Ref).

Continuous infusion: IV: Initial: 2 to 5 mcg/kg/minute; titrate based on clinical end point (eg, systemic perfusion or end-organ perfusion); usual dosage range: 2 to 10 mcg/kg/minute; maximum dose: 20 mcg/kg/minute (Ref).

Inotropic support

Inotropic support (off-label use): Note: In patients with shock (eg, sepsis) with cardiac dysfunction who fail to meet resuscitation targets despite adequate fluid status and BP, dobutamine may be added to vasopressor therapy (eg, norepinephrine) (Ref).

Continuous infusion: IV: Initial: 2 to 5 mcg/kg/minute; titrate based on clinical end point (eg, BP or other measure of perfusion); usual dosage range: 2 to 10 mcg/kg/minute; however, doses as low as 0.5 mcg/kg/min have been used in less severe cardiac decompensation; maximum dose: 20 mcg/kg/minute (Ref).

Stress echocardiography, routine

Stress echocardiography, routine (diagnostic agent) (off-label use): Continuous infusion: IV: Initial: 5 mcg/kg/minute; increase at 3-minute intervals to 10 mcg/kg/minute, then 20 mcg/kg/minute, then 30 mcg/kg/minute, and then 40 mcg/kg/minute. May coadminister atropine in patients who do not achieve target heart rate (Ref).

Stress echocardiography, viability assessment

Stress echocardiography, viability assessment (diagnostic agent) (off-label use): Continuous infusion: IV: Initial: 2.5 mcg/kg/minute; increase at 5-minute intervals in 2.5 mcg/kg/minute increments until contractile response is noted, up to a maximum dose of 10 mcg/kg/minute (Ref).

Dosing: Kidney Impairment: Adult

The renal dosing recommendations are based upon the best available evidence and clinical expertise. Senior Editorial Team: Bruce Mueller, PharmD, FCCP, FASN, FNKF; Jason A. Roberts, PhD, BPharm (Hons), B App Sc, FSHP, FISAC; Michael Heung, MD, MS.

Note: In patients with CrCl <20 mL/minute receiving dobutamine continuous infusion for treatment of acute decompensated heart failure, dobutamine-induced myoclonus has been reported as a rare, but perhaps underrecognized, adverse effect (Ref).

Altered kidney function: No dosage adjustment necessary for any degree of kidney dysfunction (excreted in the urine as inactive metabolites) (Ref).

Hemodialysis, intermittent (thrice weekly): Dialysis removal unknown (some expected based on small volume of distribution); no supplemental dose or dosage adjustment necessary (Ref).

Peritoneal dialysis: Dialysis removal unknown (some expected based on small volume of distribution); no dosage adjustment necessary (Ref).

CRRT: No dosage adjustment likely to be necessary (Ref).

PIRRT (eg, sustained, low-efficiency diafiltration): No dosage adjustment likely to be necessary (Ref).

Dosing: Liver Impairment: Adult

There are no dosage adjustments provided in the manufacturer's labeling.

Dosing: Obesity: Adult

The recommendations for dosing in patients with obesity are based upon the best available evidence and clinical expertise. Senior Editorial Team: Jeffrey F. Barletta, PharmD, FCCM; Manjunath P. Pai, PharmD, FCP; Jason A. Roberts, PhD, BPharm (Hons), B App Sc, FSHP, FISAC.

Class 1, 2, and 3 obesity (BMI ≥ 30 kg/m²):

Continuous infusion: IV: Use **ideal body weight** for initial weight-based dosing, then titrate to hemodynamic effect and clinical response (Ref). During therapy, clinicians should **not** change dosing weight from one weight metric to another (ie, actual body weight to/from ideal body weight) (Ref). Refer to "Dosing: Adult" for indication-specific doses.

Note: For dobutamine stress test dosing, use **actual body weight** for all BMI categories (Ref).

Rationale for recommendations:

There is a paucity of studies evaluating the influence of obesity on dobutamine dosing or pharmacokinetics. One observational study of patients undergoing dobutamine stress testing using a weight-based protocol (and use of atropine) supports the protocolized use of actual body weight for all classes of obesity in this setting; however, this may not be applicable to the general population receiving dobutamine as a continuous infusion for inotropic support (Ref). Dobutamine has a small volume of distribution with a short half-life enabling rapid titration to the desired effect. Due to the short onset of action and small volume of distribution, rapid titration to clinical effect after initial dosing is possible (Ref).

Dosing: Older Adult

Refer to adult dosing.

Dosing: Pediatric

(For additional information see "Dobutamine: Pediatric drug information")

Hemodynamic support

Hemodynamic support: Infants, Children, and Adolescents: Continuous IV or intraosseous infusion: Initial: 0.5 to 1 mcg/kg/minute; titrate gradually every few minutes until desired response achieved; usual range: 2 to 20 mcg/kg/minute (Ref).

Dosing: Kidney Impairment: Pediatric

There are no dosage adjustments provided in the manufacturer's labeling.

Dosing: Liver Impairment: Pediatric

There are no dosage adjustments provided in the manufacturer's labeling.

Adverse Reactions

The following adverse drug reactions and incidences are derived from product labeling unless otherwise specified.

1% to 10%:

Cardiovascular: Angina pectoris (1% to 3%), chest pain (1% to 3%), increased heart rate (10%), increased systolic blood pressure (8%), palpitations (1% to 3%), premature ventricular contractions (5%)

Gastrointestinal: Nausea (1% to 3%)

Nervous system: Headache (1% to 3%)

Respiratory: Dyspnea (1% to 3%)

Frequency not defined:

Cardiovascular: Decreased blood pressure, ventricular tachycardia

Endocrine & metabolic: Decreased serum potassium

Local: Localized phlebitis

Postmarketing:

Cardiovascular: Atrial fibrillation (Wirtz 1995), bradycardia (Olszowska 2012), cardiomyopathy (Chandraprakasam 2015), coronary artery vasospasm (Yamagishi 1998), heart block (Vaidyanathan 2008), left ventricular dysfunction (takotsubo syndrome) (Mangolini 2020), torsade de pointes (Quan 2009)

Hematologic & oncologic: Eosinophilia (Maaliki 2021)

Hypersensitivity: Hypersensitivity reaction

Nervous system: Chills (Poldermans 1993), myoclonus (Noel 2022), shivering (Poldermans 1993)

Contraindications

Hypersensitivity to dobutamine or sulfites (some contain sodium metabisulfate), or any component of the formulation; hypertrophic cardiomyopathy with outflow tract obstruction (formerly known as idiopathic hypertrophic subaortic stenosis).

Canadian labeling: Additional contraindications (not in the US labeling): Pheochromocytoma.

Note: When utilized for stress testing, additional contraindications according to the American Society of Nuclear Cardiology include patients with recent (<2 to 4 days) myocardial infarction, unstable angina, severe aortic stenosis, atrial tachyarrhythmias with uncontrolled ventricular response, prior history of ventricular tachycardia, uncontrolled hypertension (>200/110 mm Hg), and aortic dissection or large aortic aneurysm (ASNC [Henzlova 2016]).

Warnings/Precautions

Concerns related to adverse effects:

- Arrhythmias: Ventricular arrhythmias, including nonsustained ventricular tachycardia and supraventricular arrhythmias, have been reported (Tisdale 1995). Observe closely for arrhythmias in patients with decompensated heart failure; sudden cardiac death has been observed

(O'Connor 1999; Pickworth 1992; Young 2000). Ensure that ventricular rate is controlled in atrial fibrillation/flutter before initiating; may increase ventricular response rate.

- BP effects: An increase in BP is more common due to augmented cardiac output, but hypotension secondarily to vasodilation may occur at higher doses.
- Heart failure complications: An increased risk of hospitalization and death has been observed with prolonged use in New York Heart Association Class III/IV heart failure patients (O'Connor 1999).
- Tachycardia: May cause dose-related increases in heart rate.
- Ventricular ectopy: May exacerbate ventricular ectopy (dose related).

Disease-related concerns:

- Aortic stenosis: Ineffective therapeutically in the presence of mechanical obstruction such as severe aortic stenosis.
- Electrolyte imbalance: Correct electrolyte disturbances, especially hypokalemia or hypomagnesemia, prior to use and throughout therapy to minimize the risk of arrhythmias (ACC/AHA/ESC [Zipes 2006]; Tisdale 1995).
- Hypovolemia: If needed, correct hypovolemia first to optimize hemodynamics.
- Active myocardial ischemia/myocardial infarction (post): Use with caution in patients with active myocardial ischemia or recent myocardial infarction; can increase myocardial oxygen demand.

Concurrent drug therapy issues:

- Monoamine oxidase inhibitors: Use with extreme caution in patients taking monoamine oxidase inhibitors; prolong hypertension may result from concurrent use.

Dosage form specific issues:

- Sodium sulfite: Product may contain sodium sulfite.

Special populations:

- Older adults: Use with caution in older adults; start at lower end of the dosage range.

Dosage Forms: US

Excipient information presented when available (limited, particularly for generics); consult specific product labeling. [DSC] = Discontinued product

Solution, Intravenous, as hydrochloride:

Generic: 1 mg/mL (250 mL); 4 mg/mL (250 mL); 250 mg/20 mL (20 mL); 1 mg/mL (250 mL [DSC]); 2 mg/mL (250 mL); 4 mg/mL (250 mL [DSC])

Solution, Intravenous, as hydrochloride [preservative free]:

Generic: 12.5 mg/mL (20 mL); 1 mg/mL (250 mL); 2 mg/mL (250 mL); 4 mg/mL (250 mL)

Generic Equivalent Available: US

Yes

Pricing: US

Solution (DOBUTamine HCl Intravenous)

12.5 mg/mL (per mL): \$0.30 - \$0.56

Solution (DOBUTamine-Dextrose Intravenous)

1 mg/mL 5% (per mL): \$0.11 - \$0.12

2 mg/mL 5% (per mL): \$0.19

4 mg/mL 5% (per mL): \$0.14

Disclaimer: A representative AWP (Average Wholesale Price) price or price range is provided as reference price only. A range is provided when more than one manufacturer's AWP price is available and uses the low and high price reported by the manufacturers to determine the range. The pricing data should be used for benchmarking purposes only, and as such should not be used alone to set or adjudicate any prices for reimbursement or purchasing functions or considered to be an exact price for a single product and/or manufacturer. Medi-Span expressly disclaims all warranties of any kind or nature, whether express or implied, and assumes no liability with respect to accuracy of price or price range data published in its solutions. In no event shall Medi-Span be liable for special, indirect, incidental, or consequential damages arising from use of price or price range data. Pricing data is updated monthly.

Dosage Forms: Canada

Excipient information presented when available (limited, particularly for generics); consult specific product labeling.

Solution, Intravenous, as hydrochloride:

Generic: 12.5 mg/mL (20 mL)

Administration: Adult

IV: Always administer via infusion device; administer into large vein.

May be a vesicant; avoid extravasation.

Extravasation management: If extravasation occurs, stop infusion immediately; leave cannula/needle in place temporarily but do **NOT** flush the line; gently aspirate extravasated solution, then remove needle/cannula; elevate extremity; apply dry, warm compresses; initiate phentolamine antidote in severe cases (eg, when local tissue concentration is high) in addition to supportive management (Ref).

Phentolamine: **SUBQ:** Dilute 5 to 10 mg in 10 mL 0.9% sodium chloride and administer into extravasation site as soon as possible after extravasation; if infiltrated catheter has not been removed, administer initial dose through the infiltrated catheter; may repeat in 60 minutes if patient remains symptomatic (Ref).

Alternatives to phentolamine:

Nitroglycerin topical 2% ointment: Apply a 1-inch strip to the site of ischemia to cover affected area; may repeat every 8 hours as necessary (Ref).

Terbutaline: **SUBQ:** Infiltrate extravasation area with terbutaline 1 mg using a solution of terbutaline 1 mg diluted in 10 mL 0.9% sodium chloride. May repeat dose after 15 minutes (Ref).

Preparation for Administration: Adult

Vials: Dilute 20 mL vial in ≥ 50 mL of a compatible IV solution; dilute 40 mL vial in ≥ 100 mL of a compatible IV solution. Maximum concentration: 5,000 mcg/mL (5 mg/mL).

Usual Infusion Concentrations: Adult

Note: Premixed solutions available.

IV infusion: 250 mg in 500 mL (concentration: 500 **mcg**/mL), 500 mg in 250 mL (concentration: 2,000 **mcg**/mL), **or** 1,000 mg in 250 mL (concentration: 4000 **mcg**/mL) of D5W or NS

Administration: Pediatric

Parenteral: Continuous IV infusion: Vials (concentrated solution) must be diluted prior to administration; premixed IV solutions (1,000 mcg/mL, 2,000 mcg/mL, 4,000 mcg/mL) are available. Administer as a continuous IV infusion via an infusion device. Central line administration is preferred; if central line not available, may administer for a short duration through a peripheral IV catheter placed in a large vein or via intraosseous access (Ref). Administration into an umbilical arterial catheter is **not**

recommended (Ref). Frequent monitoring of the IV catheter site is recommended to rapidly identify extravasation (Ref). Refer to institutional policies and procedures; catheter placement/size and vasopressor concentration may vary depending on institution.

Rate of infusion (mL/hour) = dose (mcg/kg/minute) × weight (kg) × 60 minutes/hour divided by the concentration (mcg/mL)

May be a vesicant; avoid extravasation. If extravasation occurs, stop infusion immediately; leave cannula/needle in place temporarily but do **NOT** flush the line; gently aspirate extravasated solution, then remove needle/cannula; elevate extremity; apply dry, warm compresses. Initiate phentolamine (or alternative antidote) in severe cases (Ref).

Preparation for Administration: Pediatric

Parenteral: Continuous IV infusion: Vial (concentrated solution): Dilute in D5W, D10W, D5¹/₂NS, D5NS, or NS to a maximum concentration of 5,000 mcg/mL (Ref). Vermont Oxford Network recommends a standard concentration of 1,000 mcg/mL or 2,000 mcg/mL for neonates (Ref); in pediatric patients, ASHP recommends standard concentrations of 1,000 mcg/mL, 2,000 mcg/mL, and 4,000 mcg/mL (Ref).

Usual Infusion Concentrations: Pediatric

Note: Premixed solutions available.

IV infusion: 1,000 **mcg**/mL, 2,000 **mcg**/mL, or 4,000 **mcg**/mL

Vesicant/Extravasation Risk

May be a vesicant.

Storage/Stability

Premixed solution: Store at 20°C to 25°C (68°F to 77°F). Pink discoloration of solution indicates slight oxidation, but no significant loss of potency.

Vials: Store intact vials at 20°C to 25°C (68°F to 77°F). Following dilution in a compatible solution, use within 24 hours.

Use: Labeled Indications

Acute decompensated heart failure: Short-term management of patients with cardiac decompensation.

Use: Off-Label: Adult

Inotropic support; Stress echocardiography (diagnostic agent)

Medication Safety Issues

Sound-alike/look-alike issues:

DOBUTamine may be confused with DOPamine

High alert medication:

The Institute for Safe Medication Practices (ISMP) includes this medication among its list of drug classes (inotropic medications, IV) which have a heightened risk of causing significant patient harm when used in error (ISMP List of High-Alert Medications in Acute Care Settings ©ISMP 2024).

Metabolism/Transport Effects

Substrate of COMT

Drug Interactions

(For additional information: Launch drug interactions program)

Note: Interacting drugs may **not be individually listed below** if they are part of a group interaction (eg, individual drugs within “CYP3A4 Inducers [Strong]” are NOT listed). For a complete list of drug interactions by individual drug name and detailed management recommendations, use the drug interactions program by clicking on the “Launch drug interactions program” link above.

Atomoxetine: May increase hypertensive effects of Sympathomimetics. Atomoxetine may increase tachycardic effects of Sympathomimetics. *Risk C: Monitor*

Beta-Blockers: May decrease therapeutic effects of DOBUTamine. *Risk C: Monitor*

Bitter Orange: May increase adverse/toxic effects of Sympathomimetics. *Risk C: Monitor*

Bornaprine: Sympathomimetics may increase anticholinergic effects of Bornaprine. *Risk C: Monitor*

Calcium Salts: May decrease therapeutic effects of DOBUTamine. *Risk C: Monitor*

Cannabinoid-Containing Products: May increase tachycardic effects of Sympathomimetics. *Risk C: Monitor*

Cocaine (Topical): May increase hypertensive effects of Sympathomimetics. Management: Consider alternatives to use of this combination when possible. Monitor closely for substantially increased blood pressure or heart rate and for any evidence of myocardial ischemia with concurrent use. *Risk D: Consider Therapy Modification*

COMT Inhibitors: May increase serum concentrations of COMT Substrates. *Risk C: Monitor*

Dihydralazine: Sympathomimetics may decrease therapeutic effects of Dihydralazine. *Risk C: Monitor*

Doxapram: Sympathomimetics may increase adverse/toxic effects of Doxapram. *Risk C: Monitor*

Doxofylline: Sympathomimetics may increase adverse/toxic effects of Doxofylline. *Risk C: Monitor*

Droxidopa: Sympathomimetics may increase hypertensive effects of Droxidopa. *Risk C: Monitor*

Esketamine (Injection): May increase adverse/toxic effects of Sympathomimetics. Specifically, the risk for elevated heart rate, hypertension, and arrhythmias may be increased. *Risk C: Monitor*

Guanethidine: May increase hypertensive effects of Sympathomimetics. Guanethidine may increase arrhythmogenic effects of Sympathomimetics. *Risk C: Monitor*

Kratom: May increase adverse/toxic effects of Sympathomimetics. *Risk X: Avoid*

Linezolid: May increase hypertensive effects of Sympathomimetics. Management: Consider initial dose reductions of sympathomimetic agents, and closely monitor for enhanced blood pressure elevations, in patients receiving linezolid. *Risk D: Consider Therapy Modification*

Solriamfetol: Sympathomimetics may increase hypertensive effects of Solriamfetol. Sympathomimetics may increase tachycardic effects of Solriamfetol. *Risk C: Monitor*

Sympathomimetics: May increase adverse/toxic effects of Sympathomimetics. *Risk C: Monitor*

Tedizolid: May increase adverse/toxic effects of Sympathomimetics. Specifically, the risk for increased blood pressure and heart rate may be increased. *Risk C: Monitor*

Thyroid Products: May increase adverse/toxic effects of Sympathomimetics. Specifically, the risk of hypertension, tachycardia, and coronary insufficiency may be increased in patients with coronary artery disease. *Risk C: Monitor*

Pregnancy Considerations

Dobutamine should not be used as a diagnostic agent for stress testing during pregnancy; use should be avoided when other options are available (ESC [Regitz-Zagrosek 2018]). Medications used for the treatment of cardiac arrest in pregnancy are the same as in the non-pregnant female. Appropriate medications should not be withheld due to concerns of fetal teratogenicity. Dobutamine use during

the post-resuscitation phase may be considered; however, the effects of inotropic support on the fetus should also be considered. Doses and indications should follow current Advanced Cardiovascular Life Support (ACLS) guidelines (AHA [Jeejeebhoy 2015]).

Breastfeeding Considerations

It is not known if dobutamine is present in breast milk.

Monitoring Parameters

BP, heart rate, ECG, hemodynamic parameters as appropriate (eg, CVP, RAP, MAP, CI, PCWP, SVR, ScvO₂ or SvO₂); intravascular volume status; kidney function; urine output.

Consult individual institutional policies and procedures.

Mechanism of Action

Dobutamine, a racemic mixture, stimulates myocardial beta₁-adrenergic receptors primarily by the (+) enantiomer and some alpha₁ receptor agonism by the (-) enantiomer, resulting in increased contractility and heart rate, and stimulates both beta₂- and alpha₁-receptors in the vasculature. Although beta₂ and alpha₁ adrenergic receptors are also activated, the effects of beta₂ receptor activation may equally offset or be slightly greater than the effects of alpha₁ stimulation, resulting in some vasodilation in addition to the inotropic and chronotropic actions (Leier 1988; Majerus 1989; Ruffolo 1987). Lowers central venous pressure and wedge pressure, but has little effect on pulmonary vascular resistance (Leier 1977; Leier 1978).

Pharmacokinetics (Adult Data Unless Noted)

Onset of action: IV: 1 to 10 minutes

Peak effect: 10 to 20 minutes

Metabolism: In tissues and hepatically (via conjugation and methylation) to inactive metabolites.

Half-life elimination: 2 minutes

Excretion: Urine (as inactive metabolites).

Patient Counseling Points

(For additional information see "Dobutamine: Patient drug information")

What is this drug used for?

- It is used to help heart function in certain patients.
- It may be given to you for other reasons. Talk with the doctor.

WARNING/CAUTION: Even though it may be rare, some people may have very bad and sometimes deadly side effects when taking a drug. Tell your doctor or get medical help right away if you have any of the following signs or symptoms that may be related to a very bad side effect:

- High or low blood pressure like very bad headache or dizziness, passing out, or change in eyesight
- A fast heartbeat
- An abnormal heartbeat that is new or worse
- Fever
- Pain and irritation where this drug goes into the body
- Signs of an allergic reaction, like rash; hives; itching; red, swollen, blistered, or peeling skin with or without fever; wheezing; tightness in the chest or throat; trouble breathing, swallowing, or talking; unusual hoarseness; or swelling of the mouth, face, lips, tongue, or throat.

Note: This is not a comprehensive list of all side effects. Talk to your doctor if you have questions.

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Brand Names: International

International Brand Names by Country

For country code abbreviations (show table)

- (AE) United Arab Emirates: Dobutamine hameln | Dobutrex;
- (AR) Argentina: Dobucard | Dobutamina | Dobutamina bioquim | Dobutamina drawer | Dobutamina fabra | Dobutamina gemepe | Dobutamina gray | Dobutamina Norgreen | Dobutamina richet | Dobutrex | E.m.c. dobutamina;
- (AT) Austria: Dobutamin | Dobutamin hameln | Inotop;
- (AU) Australia: Dobutamine | Dobutamine bc | Dobutamine claris | Dobutamine hameln | Dobutamine HCL | Dobutrex;
- (BD) Bangladesh: Dobumin | Dobutamine abbott | Dobutin;
- (BE) Belgium: Dobutamine Baxter | Dobutamine eg | Dobutamine mayne pharma (ben) | Dobutrex;
- (BG) Bulgaria: Dobutamin | Dobutamin hameln | Dobutrex;
- (BR) Brazil: Cloridrato de dobutamina | Dobtan | Dobu | Dobutal | Dobutamol | Dobutanil | Dobutariston | Dobutrex | Hibutan;
- (CH) Switzerland: Dobutamin | Dobutamin fresenius | Dobutrex;
- (CL) Chile: Dobutamina;
- (CN) China: Dobutamine | Dobutrex | Feng hai fen;
- (CO) Colombia: Autobod | Dobutamina | Dobutrex | Docarip | Dotropina | Duvig;
- (CR) Costa Rica: Corbusin | Dobutamina | Dobutamina gray;
- (CZ) Czech Republic: Dobuject | Dobutamin | Dobutamin admeda | Dobutamin hameln;
- (DE) Germany: Dobutamin | Dobutamin carinopharm | Dobutamin hameln | Dobutamin hexal;
- (DO) Dominican Republic: Dobutrex | Dotropina | Inoject;
- (EC) Ecuador: Dobutam | Dobutamina | Dobutamina sanderson;
- (EE) Estonia: Dobcard | Dobuject | Dobutamine claris | Dobutamine hameln | Dobutamine HCL | Dobutamine merck | Dobutamine mylan | Dobuthaver | Dobutrex | Mekard;
- (EG) Egypt: Dobutamine | Dobutrex | Dubuject;
- (ES) Spain: Dobucor | Dobutamina abbott | Dobutamina inibsa | Dobutamina rovi | Dobutrex;
- (FI) Finland: Dobuject | Dobutamin abbott | Dobutrex;
- (FR) France: Dobutamine | Dobutamine Aguetant | Dobutamine Baxter | Dobutamine dakota | Dobutamine merck | Dobutamine Panpharma | Dobutamine Qualimed | Dobutamine winthrop | Dobutrex;
- (GB) United Kingdom: Dobutamine | Dobutrex | Posiject;
- (GR) Greece: Dobutamine HCL | Dobutan;
- (HK) Hong Kong: Dobutamine;
- (HU) Hungary: Dobuject | Dobutamin admeda | Dobutamin hameln | Dobutamina panpharma | Dobutrex;
- (ID) Indonesia: Cardiject | Dobuca | Dobuject | Doburan | Dobutamin | Dobutamine | Dobutel | Dobutrex | Domine | Dominic | Inodex | Inotrop;

- (IE) Ireland: Dobutamine | Posiject;
- (IN) India: Cardiforce | Dobicard | Dobier s | Dobunex pf | Dobusol | Dobustat | Inoject;
- (IT) Italy: Dobutamina abbott | Dobutamina bioindustria | Dobutamina Hikma | Dobutrex | Miozac;
- (JP) Japan: Bubucin | Doburack | Dobutamine | Dobutamine hydrochloride sawai | Dobutrex | Dobux | Dobux nichiiiko | Dopmin | Doputamin | Doputamin Fuji | Doputamin h hexal | Her-carenone d | Retamex;
- (KR) Korea, Republic of: Aukomine | Dobamine | Dobuject | Doburan | Dobutamine | Dobutamine hcl and dextrose | Dobutamine hcl injection huons | Dobutrex | Myungmoon Dobutamine HCL | Toburex;
- (KW) Kuwait: Dobuject | Dobutrex;
- (LB) Lebanon: Dobutamin | Dobutrex;
- (LT) Lithuania: Dobuject | Dobutamin | Dobutamina panpharma | Dobutamine claris | Dobutamine hameln | Dobutrex;
- (LV) Latvia: Dobuject | Dobutamin | Dobutamine claris | Dobutamine mylan | Dobutrex;
- (MA) Morocco: Dobutrex;
- (MX) Mexico: Adregotec | Corbusin | Cryobutol | Dobuject | Dobutamina | Dobutamina gi tecn | Dotropina;
- (MY) Malaysia: Dobutamine | Mobitil;
- (NL) Netherlands: Dobutamine | Dobutamine CF | Dobutamine claris;
- (NO) Norway: Dobutamin | Dobutrex;
- (NZ) New Zealand: Dobutamine | Dobutamine claris | Dobutamine hameln;
- (PA) Panama: Dobutamina;
- (PE) Peru: Dobusol | Dobutamina | Dobutrex;
- (PH) Philippines: Abbott dobutamine hydrochloride | Cardease | Cardiotrex | Cardob | Dobine | Dobucard | Dobucore | Dobuject | Dobulex | Dobumarc | Dobumean | Doburan | Dobusenz | Dobutamine | Dobutamine Baxter | Dobutrex | Dobutrim | Dobuzef | Inocard | Predobut | Pusogard;
- (PK) Pakistan: Dobamin | Dobutine | Dobutrex;
- (PL) Poland: Dobuject | Dobutamin hameln | Dobutrex;
- (PR) Puerto Rico: Dobutamine | Dobutamine HCL | Dobutrex;
- (PT) Portugal: Dasomin | Dobucor | Dobutamina | Dobutamina Aps | Dobutamina Claris | Dobutamina Generis | Dobutamina genthon | Dobutamina Hikma | Dobutina | Inotrex;
- (PY) Paraguay: Dobuject | Dobutamina sanderson | E.m.c.;
- (QA) Qatar: Dobcard | Dobuject | Dobutasel | Dobuthaver;
- (RO) Romania: Dobutamin admeda | Dobutamina | Dobutamina panpharma;
- (RU) Russian Federation: Dobutamin admeda | Dobutamine | Dobutel | Dobutrex;
- (SA) Saudi Arabia: Dobuject | Dobutamin | Dobutamine;
- (SE) Sweden: Dobutamin hameln | Dobutrex;
- (SG) Singapore: Dobutamine | Dobutrex;
- (SI) Slovenia: Dobutamin | Dobutamin hameln | Inotop;
- (SK) Slovakia: Dobutamin | Dobutrex;
- (SR) Suriname: Dobutamine CF | Dobutamine hameln;
- (TH) Thailand: Cardiject | Dobucin | Dobutamine | Dobutamine abbott | Dobutamine hcl DBL | Dobutel | Dobutrex;
- (TN) Tunisia: Dobutamine mylan | Dobutamine Panpharma;
- (TR) Turkey: Dobcard | Dobutamin | Dobutasel | Dobuthaver | Mekard;
- (TW) Taiwan: Butamine | Dobuha | Dobuject | Dobutamine | Dobutamine HCL | Dobutrex | Easy-dobu | Gendobu | Utamine;
- (UA) Ukraine: Cardiject | Clebutam | Dobutamin admeda | Dobutamin ebewe | Dobutel;
- (UY) Uruguay: Dobutam | Dobutamina | Dobutamina blaufarma | Dobutamina fu | Duvig;
- (VE) Venezuela, Bolivarian Republic of: Doburan | Dobutamina | Dobutrex | Dobuxin;
- (VN) Viet Nam: Butavell;
- (ZA) South Africa: Dobutamine

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18. Expert opinion. Senior Renal Editorial Team: Bruce Mueller, PharmD, FCCP, FASN, FNKF; Jason A. Roberts, PhD, BPharm (Hons), B App Sc, FSHP, FISAC; Michael Heung, MD, MS.
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